

Topic 1: Classification

To standardize melanoma staging and improve communication among multidisciplinary teams, the 7th edition of the **American Joint Committee on Cancer (AJCC)** classification was adopted as the most appropriate system. This classification is based on robust methodological development and offers a widely accepted, practical framework for clinical use.

Key changes introduced in the 7th edition compared to previous versions include:

- **Clark's level of invasion** is no longer recommended for routine use.
- For melanomas < 1 mm in thickness, the **mitotic rate** has replaced Clark's level as a prognostic factor.
- **Lymph node micrometastases** detected by **immunohistochemistry (IHC)** are now included in the **N1 category**, reflecting their clinical significance.

Recommendation

- The **7th edition of the AJCC melanoma classification** is recommended for staging patients with cutaneous melanoma.
(Evidence Level A)

This classification system is detailed in Annex 1 of the original report.

Topic 2: Resection Margins

Evidence from one randomized controlled trial and three meta-analyses has not demonstrated a clear survival benefit for wider resection margins over more conservative ones. However, most international guidelines (with the exception of

those from the United Kingdom) recommend that resection margins should not exceed **2 cm**.

Recommendation

Lateral resection margins for **primary cutaneous melanoma** should be determined according to **tumor thickness**, as follows:

- **Melanoma in situ: 0.5 cm margin**
- **Melanoma 0.1–1.0 mm thick: 1 cm margin**
- **Melanoma 1.1–2.0 mm thick: 1–2 cm margin**
- **Melanoma >2.0 mm thick: 2 cm margin**

(Recommendation Grade B)

Regarding **resection depth**, no high-level evidence is available to define an optimal plane. The project team recommends resection **down to the fascia**, provided the fascia itself is preserved.

(Recommendation Grade C)